

M083 Stroke Ischemic Full Rule Hierarchy Tree

M083 SOURCE TREE

M083 Source Tree Roundtrip

Admission

- Admission is indicated for 1 or more of the following [A] (2) (3) (4) :
 - Acute ischemic stroke [A] with neurologic findings that warrant inpatient care, as indicated by 1 or more of the following:
 - National Institutes of Health Stroke Scale (NIHSS) score greater than 2 [B] (2) (9)
 - Evidence of hemorrhagic transformation (4) (9)
 - Altered mental status (10)
 - Dysphagia that warrants evaluation (eg, aspiration risk suspected) (2) (9)
 - Significant arm or leg weakness (eg, limiting movement against gravity) (4)
 - Aphasia (4)
 - Gait impairment (4)
 - Finding on brain imaging that requires inpatient care (eg, edema, threatened herniation) (2) (9)
 - Clinical monitoring for neurologic worsening needed (eg, large infarct, posterior circulation infarct) [C] (2) (9) (11)
 - Clinical stability unclear (eg, recurrent or worsening findings)
 - Acute ischemic stroke [A] with clinical need for inpatient care or monitoring, as indicated by 1 or more of the following:
 - Hemodynamic instability (2) (9) (12)
 - Cardiac arrhythmias of immediate concern (9) (13) (14)
 - Clinically significant cardiac or vascular disorder identified (eg, on echocardiogram, imaging) that requires inpatient care (eg, severe valvular disease, atrial myxoma, cervical artery dissection) (15)
 - Ischemic stroke due to cerebral venous thrombosis [D] (1) (16)
 - Respiratory abnormalities (eg, Hypoxemia , impaired airway protection) (2)
 - Severe hypertension (SBP greater than 180 mm Hg or DBP greater than 120 mm Hg, or greater than the 95th percentile for age, sex, and height plus 30 mm Hg in pediatric patients [E] [F]) (2) (4) (17) (18)
 - Prolonged cardiac telemetry monitoring needed (eg, beyond observation care time frame) due to concern for Dangerous arrhythmia (ie, not in effort to detect atrial fibrillation) [G]
 - Suspected vasculitis (21) (22)
 - Thrombolysis or thrombectomy performed or planned [H] (2) (23) (24)
- Alternatives include (4) (26) :
 - Outpatient evaluation and treatment in emergency department, rapid treatment site, urgent care center, or medical office
 - History and physical examination
 - Laboratory and diagnostic testing (eg, brain imaging, carotid imaging, echocardiogram) (28)
 - Outpatient antithrombotic therapy and control of risk factors (eg, hypertension, hyperlipidemia)
 - Observation. See Stroke: Ischemic: Observation Care ISC guideline as appropriate.
- Extended stay beyond goal length of stay may be needed for (2) (6) (29) (31) (34) (35) :
 - Failure to meet discharge criteria (recovery milestones within final day in Optimal Recovery Course)
 - Expect brief stay extension.
 - Major deficit or clinical deterioration (42)
 - Patient presenting with severe manifestations (eg, hemiplegia, aphasia, progression in neurologic deficits, impaired cognitive or swallowing function) may require ongoing inpatient care.
 - Anticipate use of feeding tube, continuing physical therapy, and possible mechanical ventilation.
 - Anticipate permanent feeding tube placement and possible tracheostomy in patient with severe persistent dysphagia or impaired cognitive function.
 - Expect brief to moderate stay extension.
 - Seizures (3) (16) (43)
 - Patients with cerebral venous thrombosis and those undergoing thrombolytic or catheter-directed reperfusion therapy may be at higher risk of seizure. (16) (44)
 - Anticipate parenteral and oral antiseizure therapy and possible continuous EEG monitoring.
 - Expect brief stay extension.
 - Bleeding (eg, cerebral or postprocedural)
 - Bleeding may occur after thrombolytic or antithrombotic therapy or following endovascular thrombectomy.
 - Factors that may increase the risk of hemorrhagic transformation of ischemic stroke include large infarct size, thrombocytopenia, more than 10 cerebral microbleeds on imaging, and renal impairment. (2) (45) (46)
 - Anticipate need for extended observation if bleeding occurs.
 - Expect brief to moderate stay extension.
 - Increased intracranial pressure (31)
 - Anticipate hyperosmolar agents, diuretics, hyperventilation, intracranial pressure monitoring, and possible external ventricular drainage.
 - Decompressive surgical craniectomy may be needed for select patients (eg, refractory intracranial hypertension, neurologic deterioration, brainstem compression, or ventricular obstruction). (16) (35)
 - Expect moderate to prolonged stay extension.
 - Surgical intervention (47) (48)
 - Ischemic stroke with brain edema and progressive neurologic compromise may prompt surgical decompression (eg, ventriculostomy or craniotomy).
 - Children or adults with focal cerebral arteriopathy (eg, moyamoya) may require surgical revascularization. (3) (49)
 - Expect moderate to prolonged stay extension.
 - See Craniotomy, Supratentorial ISC guideline as indicated.
 - Hospital-acquired infection (eg, urinary tract infection, pneumonia) (50) (51)
 - Poststroke pneumonia is more common among patients older than 75 years with comorbid heart failure, dysarthria, and dysphagia. (52)
 - Anticipate parenteral antibiotics and supportive care.
 - Expect brief to moderate stay extension.
 - Noninfectious poststroke fever (51)
 - Expect laboratory and radiologic evaluation for infectious and noninfectious etiologies.
 - Anticipate treatment with antipyretic medications and possible treatment of underlying cause (eg, withdrawal of possibly contributing drugs, chest physiotherapy for atelectasis).
 - Expect brief stay extension.
 - Venous thromboembolism (29) (51)
 - Anticipate local external measures, anticoagulation, and possible vena cava filter.
 - See Venous Thrombosis and Pulmonary Embolism: Common Complications and Conditions ISC for further information.
 - Expect brief to moderate stay extension.
 - Severe electrolyte abnormality (eg, hypernatremia, hyponatremia)
 - Anticipate careful fluid management and close monitoring of electrolytes and mental status.

- Expect brief stay extension.
- Respiratory failure (35) (53)
 - Intubation and mechanical ventilation may be needed for obtundation, coma (eg, GCS Calculator less than 8), agitation or combativeness, loss of protective airway reflexes, significant elevation in intracranial pressure, brain herniation, or non-neurologic respiratory complications (eg, severe aspiration pneumonia, acute respiratory distress syndrome).
 - Tracheostomy may be needed for patient with Altered mental status that is severe or persistent , ongoing need for mechanical ventilation, or failure of extubation.
- Anticipate brief stay extension.
- Myocardial ischemia (54)
 - Anticipate pharmacologic treatment and possible cardiac catheterization and intervention.
- Expect brief stay extension.
- Dangerous arrhythmia (54)
 - Anticipate telemetry monitoring, possible electrophysiologic study, and antiarrhythmic medications or ablation procedure.
- Expect brief stay extension.
- See Arrhythmia: Common Complications and Conditions ISC guideline as appropriate.
- Cerebral hyperperfusion syndrome [0] (55)
 - Anticipate ICU monitoring, transcranial duplex ultrasound, and brain imaging studies.
 - Anticipate intensive blood pressure control, prophylactic antiseizure medications, and possible hyperosmolar agents for cerebral edema.
 - Intracranial hemorrhage may occur.
 - Stay extension varies.
- Poststroke delirium (56)
 - Poststroke delirium is associated with worse poststroke function and higher mortality.
- Expect brief stay extension.
- Unstable comorbidities (heart failure, Acute kidney injury (stage 2) , Acute renal failure (stage 3 acute kidney injury)) (51)
 - Unstable comorbidities may require ongoing inpatient care.
 - Expect brief stay extension.
- Discharge planning includes [P] :
 - Assessment of needs and planning for care, including (58) (59) :
 - Develop and modify treatment plan (involving multiple providers) as needed.
 - Evaluate and address preadmission functioning as needed.
 - Evaluate and address psychosocial status issues as indicated. See Psychosocial Assessment SR for further information.
 - Evaluate and address social determinants of health (eg, housing, food). See Social Determinants of Health Screening Tool SR for further information.
 - Evaluate and address patient or caregiver preferences as indicated.
 - Identify skilled services needed at next level of care, with specific attention to:
 - Home safety assessment (60)
 - Medication management, adherence instruction, and side effects assessment (61) (62) (63)
 - Neurologic status assessment
 - Nutrition and hydration management
 - Ongoing education required (64)
 - Rehabilitation therapy or equipment coordination
 - Early identification of anticipated discharge destination; options include (59) (65) :
 - Home; considerations include:
 - Home safety assessment. See Home Safety Assessment SR for further information.
 - Patient safe to go home; examples include (66) (67) (68) :
 - Medical status stable for patient's condition
 - Functional care can safely be provided with available resources.
 - Mental status stable for patient's condition
 - Medication availability confirmed and reconciliation complete
 - Patient/caregiver education completed with written discharge instructions provided
 - Community resources identified and referrals made, as needed
 - Home care arranged, if indicated
 - Necessary medical equipment delivery arranged or available in home, if indicated
 - Necessary medical supplies ordered, or patient/caregiver can obtain, if indicated
 - Access to follow-up care
 - Self-management ability if appropriate. See Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment SR for further information.
 - Caregiver need, ability, and availability
 - Post-acute skilled care or custodial care as indicated. See Discharge Planning Tool SR for further information.
 - Transitions of care plan complete, including (59) (65) (69) :
 - Patient and caregiver education complete.
 - See Teach-Back Tool SR for further information.
 - See Stroke: Ischemic: Patient Education for Clinicians SR for further information.
 - Medication reconciliation completion includes (63) (70) :
 - Compare patient's discharge list of medications (prescribed and over-the-counter) against provider's admission or transfer orders.
 - Assess each medication for correlation to disease state or medical condition.
 - Report medication discrepancies to prescribing provider, attending physician, and primary care provider, and ensure accurate medication order is identified.
 - Provide reconciled medication list to all treating providers.
 - Confirm that patient or caregiver can acquire medication.
 - Educate patient and caregiver.
 - Provide complete medication list to patient and caregiver.
 - Importance of presenting personal medication list to all providers at each care transition, including all provider appointments
 - Reason, dosage, and timing of medication (eg, use "teach-back" techniques) (71)
 - Encourage communication between patient, caregiver, and pharmacy for obtaining prescriptions, setting up home medication delivery, and reviewing for drug-drug interactions.
 - See Medication Reconciliation Tool SR for further information.
 - Plan communicated to patient, caregiver, and all members of care team, including (72) :
 - Inpatient care and service providers
 - Primary care provider
 - All post-discharge care and service providers
 - Appointments planned or scheduled, which may include:
 - Primary care provider (73)
 - Cardiologist
 - Follow-up evaluation call. See Post-Acute Care Follow-Up Call Tool SR for further information. (74)
 - Neurologist (64)
 - Psychiatrist or rehabilitation physician (64)
 - Rehabilitation therapy services (75)
 - Specialists for management of comorbidities as needed
 - Other
 - Outpatient testing and procedure plans made, which may include:
 - Laboratory testing (76)
 - Other
 - Referrals made for assistance or support, which may include:
 - Alcohol and other drug abuse or dependence treatment
 - Community services (64)
 - Educational program (eg, chronic condition management, self-management) (73) (77)

- End-stage disease planning
- Financial, for follow-up care, medication, and transportation
- Home architectural modifications
- Self-help or support groups (64)
- Social services (eg, social programs, advance directives) (64)
- Tobacco use treatment (78) (79)
- Vocational rehabilitation (80)
- Other
- Medical equipment and supplies coordinated (ie, delivered or delivery confirmed), which may include (81):
 - Adaptive devices (eg, feeding tools, reachers) (75)
 - Ambulation devices (eg, cane, crutches, walker)
 - Antiembolic or compression stockings
 - Bath and toilet aids
 - Blood pressure cuff
 - Chair lift
 - Lift equipment
 - Pressure injury prevention equipment (64)
 - Wheelchairs and accessories
 - Other
- Post-hospital levels of admission may include:
 - Home.
 - Home healthcare. See Home Care Indications for Admission Section HC in Stroke guideline in Home Care.
 - Recovery facility care. See Recovery Facility Care Indications for Admission Section RFC in Stroke guideline in Recovery Facility Care.
 - Inpatient rehabilitation facility. See Inpatient Rehabilitation Facility (Acute Rehabilitation) Indications for Admission Section RFC in Inpatient Rehabilitation Facility (Acute Rehabilitation): Stroke guideline in Recovery Facility Care.

Discharge

Discharge Planning

- Discharge planning includes [P] :
 - Assessment of needs and planning for care, including (58) (59):
 - Develop and modify treatment plan (involving multiple providers) as needed.
 - Evaluate and address preadmission functioning as needed.
 - Evaluate and address psychosocial status issues as indicated. See Psychosocial Assessment SR for further information.
 - Evaluate and address social determinants of health (eg, housing, food). See Social Determinants of Health Screening Tool SR for further information.
 - Evaluate and address patient or caregiver preferences as indicated.
 - Identify skilled services needed at next level of care, with specific attention to:
 - Home safety assessment (60)
 - Medication management, adherence instruction, and side effects assessment (61) (62) (63)
 - Neurologic status assessment
 - Nutrition and hydration management
 - Ongoing education required (64)
 - Rehabilitation therapy or equipment coordination
 - Early identification of anticipated discharge destination; options include (59) (65):
 - Home; considerations include:
 - Home safety assessment. See Home Safety Assessment SR for further information.
 - Patient safe to go home; examples include (66) (67) (68):
 - Medical status stable for patient's condition
 - Functional care can safely be provided with available resources.
 - Mental status stable for patient's condition
 - Medication availability confirmed and reconciliation complete
 - Patient/caregiver education completed with written discharge instructions provided
 - Community resources identified and referrals made, as needed
 - Home care arranged, if indicated
 - Necessary medical equipment delivery arranged or available in home, if indicated
 - Necessary medical supplies ordered, or patient/caregiver can obtain, if indicated
 - Access to follow-up care
 - Self-management ability if appropriate. See Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment SR for further information.
 - Caregiver need, ability, and availability
 - Post-acute skilled care or custodial care as indicated. See Discharge Planning Tool SR for further information.
 - Transitions of care plan complete, including (59) (65) (69):
 - Patient and caregiver education complete.
 - See Teach-Back Tool SR for further information.
 - See Stroke: Ischemic: Patient Education for Clinicians SR for further information.
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 - Provide reconciled medication list to all treating providers.
 - Confirm that patient or caregiver can acquire medication.
 - Educate patient and caregiver.
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 - Reason, dosage, and timing of medication (eg, use "teach-back" techniques) (71)
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 - Other

- Outpatient testing and procedure plans made, which may include:
 - Laboratory testing (76)
 - Other
- Referrals made for assistance or support, which may include:
 - Alcohol and other drug abuse or dependence treatment
 - Community services (64)
 - Educational program (eg, chronic condition management, self-management) (73) (77)
 - End-stage disease planning
 - Financial, for follow-up care, medication, and transportation
 - Home architectural modifications
 - Self-help or support groups (64)
 - Social services (eg, social programs, advance directives) (64)
 - Tobacco use treatment (78) (79)
 - Vocational rehabilitation (80)
 - Other
- Medical equipment and supplies coordinated (ie, delivered or delivery confirmed), which may include (81):
 - Adaptive devices (eg, feeding tools, reachers) (75)
 - Ambulation devices (eg, cane, crutches, walker)
 - Antiembolic or compression stockings
 - Bath and toilet aids
 - Blood pressure cuff
 - Chair lift
 - Lift equipment
 - Pressure injury prevention equipment (64)
 - Wheelchairs and accessories
 - Other

Discharge Destination

- Post-hospital levels of admission may include:
 - Home.
 - Home healthcare. See Home Care Indications for Admission Section HC in Stroke guideline in Home Care.
 - Recovery facility care. See Recovery Facility Care Indications for Admission Section RFC in Stroke guideline in Recovery Facility Care.
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Optimal Recovery Course

Day	Level of Care	Clinical Status	Activity	Routes	Interventions	Medications
1	Stroke unit, ICU,; [; I;]; telemetry, or floor; Social Determinants of Health Assessment; Pediatric Readmission Risk Assessment; Adult Readmission Risk Assessment; Extended Stay Risk Assessment; Discharge planning	Clinical Indications met; [; J;]; Cerebral hemorrhage excluded	Bed rest	IV fluids; IV medications; Possible central line venous access	Neurologic checks; Check lipid levels; Neurology consultation; Laboratory studies; Cardiac monitoring; Imaging studies (ie, brain, intracranial vascular); Swallowing evaluation; Oxygen if needed to maintain saturation at 94% or greater; [; K;]; Possible endovascular therapy	Blood pressure control as indicated; [; L;]; Possible antithrombotic therapy; [; M;]; Possible thrombolytic therapy
2	Stroke unit, ICU, telemetry, or floor; Social Determinants of Health Assessment; Pediatric Readmission Risk Assessment; Adult Readmission Risk Assessment; Extended Stay Risk Assessment	Hemodynamic stability; Mental status at baseline or stable; Neurologic deficits absent or stable; Unimpaired swallowing	Up to chair; Rehabilitation at bedside; Possible assisted ambulation	Oral hydration; Oral medications; Advance diet as tolerated	Neurologic checks; Rehabilitation assessment; Possible oxygen; Possible physical therapy; Possible occupational and speech therapy	Antithrombotic therapy; Blood pressure control
3	Social Determinants of Health Assessment; Pediatric Readmission Risk Assessment; Adult Readmission Risk Assessment; Extended Stay Risk Assessment; Stroke unit or floor to discharge; Complete discharge planning	Hemodynamic stability; Dangerous arrhythmia absent; Mental status at baseline or stable and appropriate for next level of care; Neurologic deficits absent or stable and appropriate for next level of care; Discharge plans and education understood	Ambulatory or acceptable for next level of care	Oral hydration; [; N;]; Oral medications or regimen acceptable for next level of care; Oral diet or acceptable for next level of care; Advance diet as tolerated	Supplemental oxygen absent or at baseline requirement; Neurologic checks; Possible physical therapy; Possible occupational and speech therapy	Antithrombotic therapy appropriate for next level of care established; [; M;]; Statin; Blood pressure control
(; 1;); (; 2;); (; 6;); (; 31;); (; 34;); (; 35;); (; 39;); (; 40;)						

DOMAIN RULE TREE

M083 Domain Rule Tree

Admission

- [OR] Admission is indicated for 1 or more of the following
 - Path: Acute ischemic stroke with neurologic findings that warrant inpatient care, as indicated by 1 or more of the following
 - [AND] Acute ischemic stroke with neurologic findings that warrant inpatient care, as indicated by 1 or more of the following [context] Acute ischemic stroke
 - [OR] Acute ischemic stroke with neurologic findings that warrant inpatient care, as indicated by 1 or more of the following
 - [atomic > NIHSS] National Institutes of Health Stroke Scale (NIHSS) score greater than 2))
 - [atomic] Evidence of hemorrhagic transformation))
 - [atomic] Altered mental status)
 - [atomic] Dysphagia that warrants evaluation (eg, aspiration risk suspected)))
 - [atomic] Significant arm or leg weakness (eg, limiting movement against gravity)))
 - [atomic] Aphasia)
 - [atomic] Gait impairment)
 - [atomic] Finding on brain imaging that requires inpatient care (eg, edema, threatened herniation)))
 - [atomic] Clinical monitoring for neurologic worsening needed (eg, large infarct, posterior circulation infarct))))
 - [atomic] Clinical stability unclear (eg, recurrent or worsening findings)
 - Path: Acute ischemic stroke with clinical need for inpatient care or monitoring, as indicated by 1 or more of the following
 - [AND] Acute ischemic stroke with clinical need for inpatient care or monitoring, as indicated by 1 or more of the following

[context] Acute ischemic stroke
 [OR] Acute ischemic stroke with clinical need for inpatient care or monitoring, as indicated by 1 or more of the following
 [atomic] Hemodynamic instability)))
 [atomic] Cardiac arrhythmias of immediate concern)))
 [atomic] Clinically significant cardiac or vascular disorder identified (eg, on echocardiogram, imaging) that requires inpatient care (eg, severe valvular disease, atrial myxoma, ce
 [atomic] Ischemic stroke due to cerebral venous thrombosis))
 [atomic] Respiratory abnormalities (eg, Hypoxemia , impaired airway protection))
 [atomic] Prolonged cardiac telemetry monitoring needed (eg, beyond observation care time frame) due to concern for Dangerous arrhythmia
 (ie, not in effort to detect atrial fibrillat
 [atomic] Suspected vasculitis))
 [OR] Severe hypertension (SBP greater than 180 mm Hg or DBP greater than 120 mm Hg, or greater than the 95th percentile for age, sex, and height plus 30 mm Hg in pediatric patie
 [atomic] > SBP) Systolic blood pressure greater than 180 mm Hg
 [atomic] > DBP) Diastolic blood pressure greater than 120 mm Hg
 [atomic] Severe hypertension (SBP greater than 180 mm Hg or DBP greater than 120 mm Hg, or greater than the 95th percentile for age, sex, and height plus 30 mm Hg in pediatric patie
 - Path: Thrombolysis or thrombectomy performed or planned
 [atomic] Thrombolysis or thrombectomy performed or planned)))

Discharge

- [CHECKLIST] Discharge planning includes
 [CHECKLIST] Discharge planning includes
 [CHECKLIST] Assessment of needs and planning for care, including))
 [atomic] Develop and modify treatment plan (involving multiple providers) as needed.
 [atomic] Evaluate and address preadmission functioning as needed.
 [atomic] Evaluate and address psychosocial status issues as indicated. See Psychosocial Assessment SR for further information.
 [atomic] Evaluate and address social determinants of health (eg, housing, food). See Social Determinants of Health Screening Tool SR for further information.
 [atomic] Evaluate and address patient or caregiver preferences as indicated.
 [CHECKLIST] Identify skilled services needed at next level of care, with specific attention to
 [atomic] Home safety assessment)
 [atomic] Medication management, adherence instruction, and side effects assessment)))
 [atomic] Neurologic status assessment
 [atomic] Nutrition and hydration management
 [atomic] Ongoing education required)
 [atomic] Rehabilitation therapy or equipment coordination
 [OPTIONS] Early identification of anticipated discharge destination; options include))
 [CHECKLIST] Home; considerations include
 [atomic] Home safety assessment. See Home Safety Assessment SR for further information.
 [EXAMPLE_SET] Patient safe to go home; examples include)))
 [atomic] Medical status stable for patient's condition
 [atomic] Functional care can safely be provided with available resources.
 [atomic] Mental status stable for patient's condition
 [atomic] Medication availability confirmed and reconciliation complete
 [atomic] Patient/caregiver education completed with written discharge instructions provided
 [atomic] Community resources identified and referrals made, as needed
 [atomic] Home care arranged, if indicated
 [atomic] Necessary medical equipment delivery arranged or available in home, if indicated
 [atomic] Necessary medical supplies ordered, or patient/caregiver can obtain, if indicated
 [atomic] Access to follow-up care
 [atomic] Self-management ability if appropriate. See Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment SR for further information.
 [atomic] Caregiver need, ability, and availability
 [atomic] Post-acute skilled care or custodial care as indicated. See Discharge Planning Tool SR for further information.
 [CHECKLIST] Transitions of care plan complete, including)))
 [CHECKLIST] Patient and caregiver education complete.
 [atomic] See Teach-Back Tool SR for further information.
 [atomic] See Stroke: Ischemic: Patient Education for Clinicians SR for further information.
 [CHECKLIST] Medication reconciliation completion includes))
 [atomic] Compare patient's discharge list of medications (prescribed and over-the-counter) against provider's admission or transfer orders.
 [atomic] Assess each medication for correlation to disease state or medical condition.
 [atomic] Report medication discrepancies to prescribing provider, attending physician, and primary care provider, and ensure accurate medication order is identified.
 [atomic] Provide reconciled medication list to all treating providers.
 [atomic] Confirm that patient or caregiver can acquire medication.
 [CHECKLIST] Educate patient and caregiver.
 [atomic] Provide complete medication list to patient and caregiver.
 [atomic] Importance of presenting personal medication list to all providers at each care transition, including all provider appointments
 [atomic] Reason, dosage, and timing of medication (eg, use "teach-back" techniques))
 [atomic] Encourage communication between patient, caregiver, and pharmacy for obtaining prescriptions, setting up home medication delivery, and reviewing for drug-drug interactions.
 [atomic] See Medication Reconciliation Tool SR for further information.
 [CHECKLIST] Plan communicated to patient, caregiver, and all members of care team, including))
 [atomic] Inpatient care and service providers
 [atomic] Primary care provider
 [atomic] All post-discharge care and service providers
 [OPTIONS] Appointments planned or scheduled, which may include
 [atomic] Primary care provider)
 [atomic] Cardiologist
 [atomic] Follow-up evaluation call. See Post-Acute Care Follow-Up Call Tool SR for further information.)
 [atomic] Neurologist)
 [atomic] Psychiatrist or rehabilitation physician)
 [atomic] Rehabilitation therapy services)
 [atomic] Specialists for management of comorbidities as needed
 [atomic] Other
 [OPTIONS] Outpatient testing and procedure plans made, which may include
 [atomic] Laboratory testing)
 [atomic] Other
 [OPTIONS] Referrals made for assistance or support, which may include
 [atomic] Alcohol and other drug abuse or dependence treatment
 [atomic] Community services)
 [atomic] Educational program (eg, chronic condition management, self-management)))

- [atomic] End-stage disease planning
- [atomic] Financial, for follow-up care, medication, and transportation
- [atomic] Home architectural modifications
- [atomic] Self-help or support groups)
- [atomic] Social services (eg, social programs, advance directives))
- [atomic] Tobacco use treatment))
- [atomic] Vocational rehabilitation)
- [atomic] Other
- [OPTIONS] Medical equipment and supplies coordinated (ie, delivered or delivery confirmed), which may include)
 - [atomic] Adaptive devices (eg, feeding tools, reachers))
 - [atomic] Ambulation devices (eg, cane, crutches, walker)
 - [atomic] Antiembolic or compression stockings
 - [atomic] Bath and toilet aids
 - [atomic] Blood pressure cuff
 - [atomic] Chair lift
 - [atomic] Lift equipment
 - [atomic] Pressure injury prevention equipment)
 - [atomic] Wheelchairs and accessories
 - [atomic] Other
- Discharge destination
 - [OPTIONS] Post-hospital levels of admission may include
 - [atomic] Home.
 - [atomic] Home healthcare. See Home Care Indications for Admission Section HC in Stroke guideline in Home Care.
 - [atomic] Recovery facility care. See Recovery Facility Care Indications for Admission Section RFC in Stroke guideline in Recovery Facility Care.
 - [atomic] Inpatient rehabilitation facility. See Inpatient Rehabilitation Facility (Acute Rehabilitation) Indications for Admission Section RFC in Inpatient Rehabilitation Facility (

LINKED RULE TREE

M083 Linked Rule Tree

Admission path: Acute ischemic stroke with neurologic findings that warrant inpatient care

- altered_mental_status_condition_present
 - **linked shared definition:** comp.altered_mental_status.root
 - source: Definition - Altered mental status

Admission path: Acute ischemic stroke with clinical need for inpatient care or monitoring

- hemodynamic_instability_condition_present
 - **linked shared definition:** comp.hemodynamic_instability.root
 - source: Definition - Hemodynamic instability
 - preview (subset):
 - SBP < 80 mmHg
 - shock_index > 1.0 ratio
 - MAP < 65 mmHg
 - vasopressor_or_inotrope_required
 - lactate >= 2.0 mmol/L
 - pH < 7.35

Admission path: Thrombolysis or thrombectomy performed or planned

No linked shared definitions on this path.

SHARED CONDITION DEFINITIONS

M083 Shared Condition Definitions

altered_mental_status_condition_present

Source: Definition - Altered mental status

OR

- confusional_state_condition_present
- lethargy_condition_present
- obtundation_condition_present
- stupor_condition_present
- coma_condition_present

hemodynamic_instability_condition_present

Source: Definition - Hemodynamic instability

OR

- Vital sign abnormality not corrected by appropriate treatment, as indicated by 1 or more of the following

OR

- Tachycardia that persists despite appropriate treatment (eg, treatment of underlying cause, despite observation care)

AND

- Tachycardia, [A][B] as indicated by 1 or more of the following

OR

- heart_rate > 100 bpm
- heart_rate > 85 bpm _(needs_review)_
- heart_rate > 95 bpm _(needs_review)_
- heart_rate > 110 bpm _(needs_review)_
- heart_rate > 120 bpm _(needs_review)_
- heart_rate > 150 bpm _(needs_review)_
- persistence_despite_appropriate_treatment_context_present

- Hypotension that persists despite appropriate treatment (eg, treatment of underlying cause, despite observation care)

AND

- Hypotension, as indicated by 1 or more of the following

OR

- Hypotension in adult patient, as indicated by 1 or more of the following

OR

- SBP < 90 mmHg
- MAP < 70 mmHg
- baseline_sbp_decrease_with_symptoms_condition_present

- Hypotension in pediatric patient, as indicated by 1 or more of the following

OR

- SBP < 110 mmHg _(needs_review)_
- SBP < 100 mmHg _(needs_review)_
- SBP < 95 mmHg _(needs_review)_
- SBP < 90 mmHg _(needs_review)_
- SBP < 80 mmHg _(needs_review)_
- SBP < 70 mmHg _(needs_review)_
- SBP < 65 mmHg _(needs_review)_
- persistence_despite_appropriate_treatment_context_present

- Orthostatic hypotension that persists despite appropriate treatment (eg, treatment of underlying cause, despite observation care)

AND

- Orthostatic hypotension, [A][B] as indicated by 1 or more of the following(1)(2)(3)

OR

- orthostatic_SBP_drop >= 20 mmHg
- orthostatic_DBP_drop >= 10 mmHg
- persistence_despite_appropriate_treatment_context_present

- Severe hypotension in adult patient, as indicated by 1 or more of the following

OR

- SBP < 80 mmHg
- shock_index > 1.0 ratio
- MAP < 65 mmHg
- vasopressor_or_inotrope_required
- Hypoperfusion due to hypotension, as indicated by ALL of the following

AND

- Hypotension, as indicated by 1 or more of the following

OR

- Hypotension in adult patient, as indicated by 1 or more of the following

OR

- SBP < 90 mmHg
- MAP < 70 mmHg
- baseline_sbp_decrease_with_symptoms_condition_present

- Hypotension in pediatric patient, as indicated by 1 or more of the following

OR

- SBP < 110 mmHg _(needs_review)_
- SBP < 100 mmHg _(needs_review)_
- SBP < 95 mmHg _(needs_review)_
- SBP < 90 mmHg _(needs_review)_
- SBP < 80 mmHg _(needs_review)_
- SBP < 70 mmHg _(needs_review)_
- SBP < 65 mmHg _(needs_review)_
- Evidence of hypoperfusion, as indicated by 1 or more of the following

OR

- lactate >= 2.0 mmol/L
- pH < 7.35
- end_organ_dysfunction_due_to_hypotension_condition_present

- Severe hypotension in pediatric patient, as indicated by 1 or more of the following

OR

- pediatric_systolic_blood_pressure_threshold_condition_present _(needs_review)_
- pediatric_shock_index_threshold_condition_present _(needs_review)_
- pediatric_vasopressor_or_inotrope_required _(needs_review)_
- pediatric_hypoperfusion_due_to_hypotension_condition_present _(needs_review)_

hypotension_condition_present

Source: Definition – Hypotension

OR

- Hypotension in adult patient, as indicated by 1 or more of the following

OR

- SBP < 90 mmHg
- MAP < 70 mmHg
- baseline_sbp_decrease_with_symptoms_condition_present

- Hypotension in pediatric patient, as indicated by 1 or more of the following

OR

- SBP < 110 mmHg _(needs_review)_
- SBP < 100 mmHg _(needs_review)_
- SBP < 95 mmHg _(needs_review)_
- SBP < 90 mmHg _(needs_review)_
- SBP < 80 mmHg _(needs_review)_
- SBP < 70 mmHg _(needs_review)_
- SBP < 65 mmHg _(needs_review)_

orthostatic_hypotension_condition_present

Source: Definition - Orthostatic hypotension

OR

- orthostatic_SBP_drop >= 20 mmHg
- orthostatic_DBP_drop >= 10 mmHg

tachycardia_condition_present

Source: Definition - Tachycardia

OR

- heart_rate > 100 bpm
- heart_rate > 85 bpm _(needs_review)_
- heart_rate > 95 bpm _(needs_review)_
- heart_rate > 110 bpm _(needs_review)_
- heart_rate > 120 bpm _(needs_review)_
- heart_rate > 150 bpm _(needs_review)_